Tucatinib + capecitabine + trastuzumab

Tucatinib twice daily every day; capecitabine twice daily on Days 1–14 then off on Days 15–21; IV trastuzumab every 21 days. Continue while beneficial and tolerated, with clinician-approved changes.

Draft — independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Diarrhoea / dehydration

Tucatinib + capecitabine

Report diarrhoea early. Follow the separate tablet-hold instructions; reduced urine or dizziness needs prompt advice.

3 Liver injury

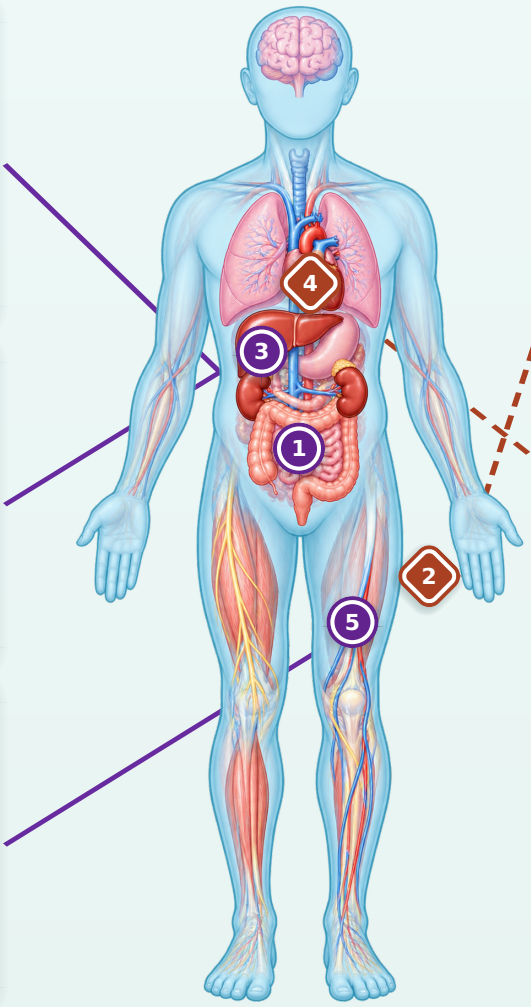
Tucatinib

Keep three-weekly liver blood tests. Report yellow skin / eyes or dark urine.

5 Blood counts / infection

Capecitabine

Fever, shivers or sudden illness: call promptly. Blood tests guide the next cycle.



2 Hands / feet

Capecitabine

Painful redness or peeling affecting activity: stop capecitabine and contact oncology.

4 Heart / chest symptoms

Trastuzumab / capecitabine

Keep echoes. New chest pain: stop capecitabine and seek urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

NCCP 00936 · PRACTICAL SUPPORT

Your treatment, at home.

Important medicine-specific effects

Tucatinib

- Diarrhoea and dehydration
- Liver test changes or liver injury
- Nausea and fatigue
- Clinically important medicine interactions

Capecitabine

- Diarrhoea / dehydration
- Hand-foot syndrome
- Mouth ulcers / reduced intake
- Low blood counts and infection
- Cardiotoxicity including chest pain / ischaemia
- Severe early toxicity including DPD-related toxicity
- Renal impairment and medicine interactions

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Support for this phase

Obtain a clear diarrhoea action plan and separate instructions for each tablet. Anti-sickness medicines and interactions require individual review.

Bloods and cardiac checks

FBC, renal, liver; DPD testing before capecitabine → Extra tests if symptoms / toxicity → Pre-cycle bloods and symptom review. Symptoms need assessment between visits.

Visits and practical support

Ask about travel, work, fertility, menopause, exercise and help at home. The team arranges and reviews clinical checks; the patient should not have to calculate the monitoring timetable.

Follow-up and bone health

If an aromatase inhibitor or ovarian suppression is prescribed, assess fracture risk and baseline DEXA as indicated. Repeat timing depends on the result, risk and bone treatment; it is not an automatic scan for everyone.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00936/

Sources and review status are available online. Follow your own prescription and hospital instructions.